Final Night

9 Nov 2025 | 9:00 AM



RS

King Abdul Aziz University, Jeddah

22252

🔍 فهرس | Contents

Physiology	Page 5
Pathology	Page 17
Medicine	Page 52
Radiology	Page 56
Pharmacology	Page 65
Parasitology	Page 99

لولا الله ثم تعب وتعاون هذا الفريق لم يكن ليُكتب لهذا العمل الظهور بهذا الشكل

MED24 TEAM

أهيل الزهراني
جود الجهني
يزيد المجنوني
ندى النمر
عزيزه باداود
سلمان السنوسي
رهام صهلولي
غدى المالكي
غيداء الحربي
ميسان كلنتن
لينه السلمي

رؤوم الغامدي
نورا القصيمي
هاشم المكرمي
طلال النافعي
رواء حبيب الله
رؤى الوافي
ولاء النمري
نايف العتيبي
محمد المعيرفي
ارينا القرني
ليان الجدعاني

ريم البلادي
إسراء أبوخماس
تركي حجي
ابوبكر الكثيري
أديب أبوشال
تاليا نوع
رتال البادي
معاذ الحارثي
محمد ظافر الشهري
عبدالرحمن باويان

لينه العطاس
محمد علي الشهري
سفانا الوذيناني
وجد الشهري
سارا الصاعدي
سامر المرغلاني
محمد شراحيلي
جواد العبيدي
ميس عامر
رنا السنوسي
ابراهيم الجهني
رزان احمد

شكراً لكم

محمد سندي

أماني الشمراني

بيان المالكي

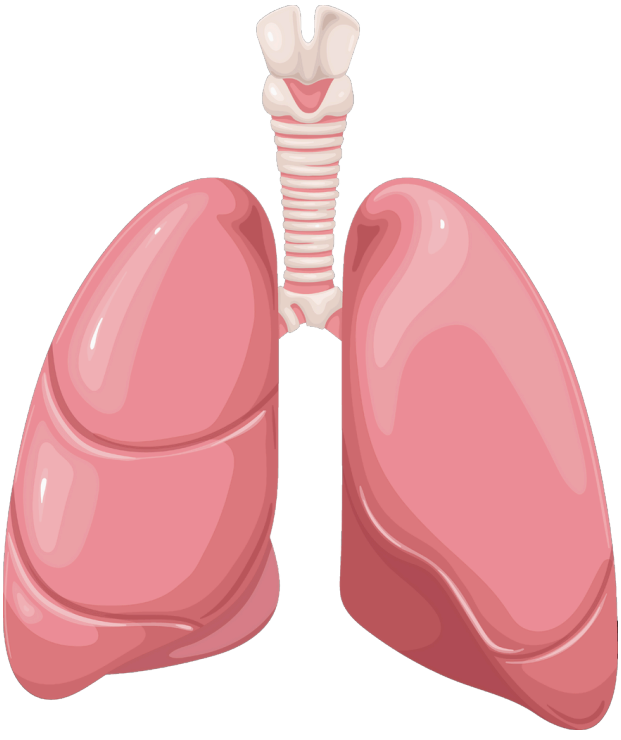
تحت اشراف:

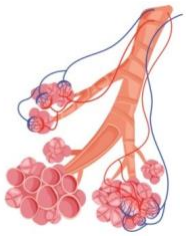
ريان الغامدي

ليان الحربي

٢٠٢٥/١٠/٢٧

اللهم إني أسألك فهم النبيين
وحفظ الملائكة المقربين،
اللهم اجعل لساني عامرًا بذكرك
وقلبي بخشيتك،
وسري بطاعتك،
إنك على كل شيء قدير
وحسبنا اللّٰه ونعم الوكيل



PHYSIOLOGY 

L6- Neural control of breathing

(14 Q's)

1- Which receptors are located on the alveolar walls, close to the pulmonary capillaries?

A. J receptors
B. Proprioceptors
C. Stretch receptors
D. Irritant receptors

Ans: A

Female slide 17

2- Which of the following won't produce apnea?

A. Exercise
B. Increase blood pressure
C. Signal from cerebral cortex
D. Swallowing

Ans: A

3- Hyperinflation of the lung can result in which of the following ?

A. Contraction of the diaphragm
B. Stimulation of the ventral respiratory neurons
C. Sneezing and coughing
D. Stimulate vagal impulses to the medullary centers

Ans: D

Male slide 21

4- A patient is experiencing hyperventilation. Which of the following receptors is not normally active during quiet breathing but becomes active to help decrease ventilation?

A. Ventral respiratory group
B. Dorsal respiratory group
C. Apneustic center
D. Pneumotaxic Center

Ans: A

Female slide 5

5- Which of the following limits inspiration?

A. Ventral respiratory group
B. Lung stretch receptors

C. Irritant receptor

D. Central chemoreceptor

Ans: B

Male slide 21

6- Which of the following neurons limit the extent of lung inflation?

A. Stretch lung receptor

B. Irritant receptor

C. J receptors

D. Chemoreceptors

Ans: A

Male slide 21

7- Which pattern occurs in mid-pontine transection with the vagi intact?

A. Tachypnea

B. Normal respiratory breathing

C. Apneustic breathing

D. Deep and slow respiration

Ans: D

Male slide 15

8- Which signal is released to limit lung inflation ?

A. Lung stretch receptor

B. Lung irritant receptor

C. Arterial baroreceptor

D. Atrial receptor

Ans: A

Male slide 21

9- A patient with a stroke results in blocking blood supply from the medulla center , which of the following will be the outcome ?

A. Immediate cessation of breath

B. Apneustic breathing

C. Ataxia breathing

D. Irregular breathing

Ans: A

Male slide 13&14

10- A 24-year-old male was playing American football and collided head-to-head with another player. He was brought to the ER with cyanosis and no respiration, requiring ventilation support. Which of the following spinal levels is most likely affected?

A. C1

B. C5

C. C8

D. T2

Ans:A

Male slide 14

11- Which reflex protects the respiratory tract during coughing?

A. Pharyngeal receptor

B. Irritant receptor

C. Central receptor

D. Peripheral receptor

Ans: B

Male slide 20

12- A 54 year old man with a history of hypertension he missed to receive his anti hypertensive for days , but there was a decrease in the pressure noticed .

What cause this decrease in pressure?

A. Cerebral cortex

B. Carotid body

C. Arterial baroreceptor

D. Proprioreceptor

Ans: C

Med21 female slide 25

13- What is the role of the neurons in the ventral respiratory group?

A. Regulate tidal volume
B. Control rhythmic breathing
C. Affect accessory muscles of respiration
D. Mediate gas exchange in the lungs

Ans: C

Male slide 6

14- Patient had a previous stroke , she got a second one which stopped blood supply to the medullary region what will occur?

A. apneustic breathing
B. Immediate cessation of breath
C. Ataxic breathing
D. Tachypnea

Ans: B

Male slide 5&6

L7- Physiology Chemical Control of Breathing

(6 Q's)

1- Peripheral chemoreceptors are most sensitive when partial pressure of oxygen falls below?

A. 100

B. 60

C. 0

D. 30

Ans: B

Edit male slide 7

2- Perephiral chemoreseptors?

A. React to hypocapnia more rapid than central chemoreceptors

B. Don't react to hypocapnia

C. Don't react to hypercapnia

D. react to increased pH

Ans: B

Male slide 3

3- Which of the following can be stimulus in response to lactic acid?

A. Central chemoreceptors
B. Peripheral chemoreceptors
C. Lung irritate receptors
D. Arterial baroreceptors

Ans: B

Male slide 3

4 - Which of the following true regarding central chemoreceptor?

A. Maximum discharge responding to H in CSF
B. Respond directly to increase CO2
C. Respond to changes in ph
D. Sensitive to hypoxia

Ans: A

Male slide 10

5 - An athletic goes to gem after training the lactic acid in his body elevate which of the following can be stimulus in response to lactic acid?

A. Central chemoreceptors
B. Peripheral chemoreceptors

C. Lung irritant receptors

D. Arterial baroreceptors

Ans: B

Male slide 3

6 - What is true about central receptors?

A. CO2 directly activates them

B. Maximum stimulation by increased H+ in CSF

C. Affected by arterial pH

D. Increase H+ in plasma stimulate it

Ans: B

Male slide 10

L8- Hypoxia and high altitude

(5 Q's)

1 - Hormone produced in excess one month after climbing a mountain?

A. Tyroxin

B. Natruetic peptide

C. Adrenaline

D. Erythropoietin

Ans: D

Edit female slide 16

2 - Patient was brought to emergency because he have hypoxia and distress,his oxygen pressure decreases markedly and not improved by oxygen therapy, what's the cause?

A. Ventilation-perfusion mismatch
B. Defusion defect
C. Hypoventilation
D. Toxicity of mitochondrial enzyme

Ans: D

Edit female slide 17

3 - A person arrives to the ER with tachycardia and tachypnea. It was noted that the patient was hypoxic and was given O2 therapy. The patient did not respond to therapy. Which of the following defects could contribute to this?

A. Congenital anomalies
B. Histotoxic
C. High altitude
D. Ventilation \ perfusion ration dismatch

Ans: B

Edit female slide 6

4 - Which of the following will not increase within a day of climbing a mountain?

A. Heart rate

B. Hematocrit

C. 2,3- DBG

D. pH

Ans: B

Edit female slide 15

5- A person arrives to the ER with tachycardia and tachypnea. It was noted that the patient was hypoxic and was given O2 therapy. The patient did not respond to therapy. Which of the following defects could contribute to this?

A. Ventricular septal defect

B. Mitochondiral enzyme defect

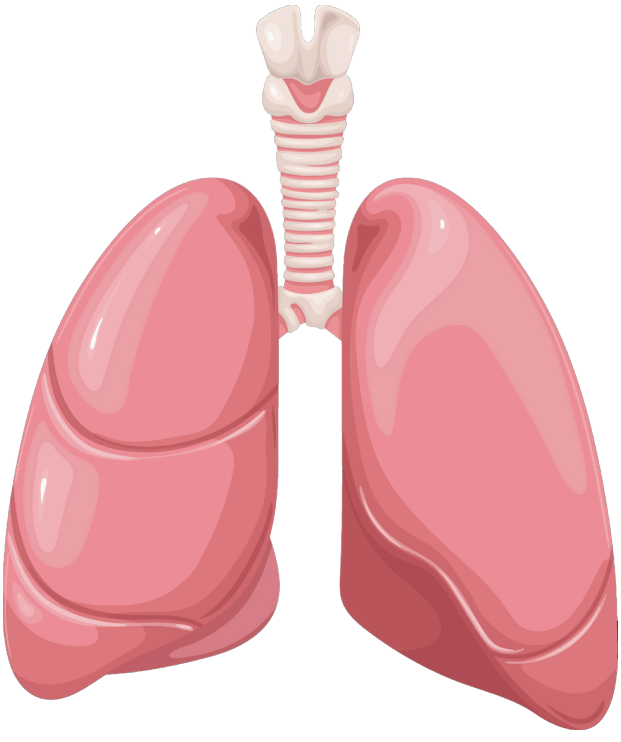
C. CO poisoning

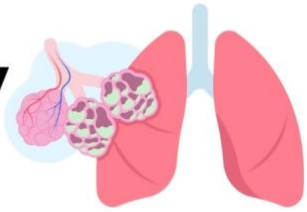
D. V/Q imbalance

Ans: B

Edit female slide 17

اللهم افتح لي ابواب حكمتك،
وانشر علي رحمتك،
وآمن علي بالحفظ والفهم،
سبحانك لا علم لنا الا ماعلمتنا،
انك انت العليم الحكيم .



PATHOLOGY 

L2- COPD

(17Q's)

1- Cigarettes smoking contribute to pathogenesis of emphysema by all except?

A. Stimulation of macrophages elastase activity

B. Stimulate release of neutrophil elastase

C. Attracts neutrophils to the lung

D. Inhibit the ability of the pulmonary neutrophil to destroy the bacteria

Ans: D

Female slide 18 and 19

2- Decrease alpha 1 anti-trypsin occurs in?

A. Distal Acinar

B. Pan-acinar

C. Centri-acinar

D. Para-septal acinar

Ans: B Female slide 14

3- An old man who is a chronic smoker had a progressively increasing dyspnea, he had barrel chest appearance and had histological changes more in his upper lobes, his condition is most likely due to which of the following?

A. Due to mucous plug

B. Due to release of elastase from neutrophils

C. Due to Alpha-1-antitrypsin deficiency

D. Due to cystic fibrosis

Ans: B

Female slide 18

4- Which of the following diseases cause a decreased FEV1/FVC?

A. Chronic Bronchitis

B. Pneumoconiosis

C. Acute interstitial lung disease

D. Sarcoidosis

Ans: A

Female slide 6

5- What type of emphysema associated with antitrypsin deficiency?

A. Distal Acinar

B. Pan-acinar

C. Centri-acinar

D. Para-septal acinar

Ans: B

Female slide 14

6- A 58 years old patient has obesity, loud snoring at night, deveolped cor pumlonae what is the most likely cause of his condition?

A. Emphysema

B. Lung tumor

C. Hypoxia

D. Infection

Ans: C

Female slide 10

7- Redix index?

A. The ratio of thickness of submucosal gland to bronchial wall
B. The ratio of thickness of cartilage to bronchial wall
C. The ratio of thickness of mucosa to bronchial wall
D. The ratio of thickness of muscular part to bronchial wall

Ans: A

Female slide 30

8- A 67 years old male, two-packed a day smoker since age 18, has had a productive cough above the last 20 years, his symptoms worsens the last 4 years, Arterial pO2 is decreased Arterial pCO2 is increased, total lung capacity is normal, these finding are most suggestive of which pulmonary disease?

A. Adult respiratory distress syndrome
B. Bronchial Asthma
C. Chronic bronchitis
D. Panacinar emphysema

Ans: C

Female slide 9

9- A 56 year old man came to the clinic complaining of weight loss and progressive dyspnea, the doctor noticed that the patient shows pursed lip breathing. What is the histological feature of the underlying disease ?

A. Dilation of bronchi with destruction of their wall

B. Bronchial mucus glands hyperplasia

C. Alveolar damage with proteinaceous fluid in alveolar space

D. Destruction and enlargement of alveolar walls

Ans: D

Female slide 31

10- (Case) Patient have a productive cough with purulent septum & situs inversus?

A. Bronchectasis

B. Emphysema

C. Atelectasis

D. Chronic bronchitis

Ans: A

Female slide 27

11- A 40-year-old female had suffered from multiple attacks of necrotising pneumonia. She now has a productive cough of a large amount of offensive purulent sputum. The chest X-ray suggested dilatation of the right lower lobe bronchi. Which of the following mechanisms is the cause of bronchial dilatation?

A. Unopposed action of neutrophil-derived elastase

B. Damage to bronchial mucosa by inflammatory mediators released from eosinophils

C. Destruction of bronchial walls by recurrent inflammation with fibrosis

D. Congenital weakness of supporting structures of the bronchial wall

And: C

Female slide 26

12- 86 years old woman has difficulty and excreting the air all the time. She has no history of smoking or being exposed to fumes. There is no history of weight loss and no persistent cough. The most common reason for her condition is?

A. lose the elasticity of the lung

B. Bronchial asthma

C. Lung cancer

D. Decrease surfactant

Ans: A

13- A 52-year-old female patient presents to the clinic with progressive shortness of breath, fatigue, and a chronic cough. Laboratory tests reveal elevated liver enzymes (ALT and AST). A diagnosis of cirrhosis is made. Which of the following types of emphysema is most likely present in this scenario?

A. Centriacinar emphysema

B. Panacinar emphysema

C. Paraseptal emphysema

D. Irregular emphysema

Ans: B Female slide 13

14- A 35-year-old male patient presents to the clinic for a routine check-up. He has no significant past medical history and denies any symptoms. Physical examination, including auscultation of the chest, reveals normal breath and heart sounds. However, a routine chest X-ray shows situs inversus. Given the patient's clinical history and imaging findings, what is the most likely diagnosis?

A. Asthma

B. COPD

C. Bronchiectasis

D. Pulmonary fibrosis

Ans: C

Female slide 26

15- Reid index?

A. The ratio of thickness of submucosal gland to bronchial wall

B. The ratio of thickness of cartilage to bronchial wall

C. The ratio of thickness of mucosa to bronchial wall

D. The ratio of thickness of muscular part to bronchial wall

Ans: A

Female slide 8

16- Which of the following types of emphysema are associated with smoking?

A. Panacinar emphysema

B. Centriacinar emphysema

C. Irregular emphysema

D. Paraseptal emphysema

Ans: B

Female slide 12

17- A 20-year-old male died. At the age of 15, he started showing shortness of breath and hepatic cirrhosis. What is the underlying cause of his condition?

A. Decrease in α_1-antitrypsin

B. Decrease in elastase

C. Decrease in protease

Ans: A

Female slide 13

L3 - Pathology of bronchial asthma

(12 Q's)

1- What does NOT complicate Bronchial asthma?

A. Pneumonia

B. Recurrent infections

C. Panacinar emphysema

D. Hyperplasia of goblet cells

Ans: C

Male slide 23

2- Status asthmatics?

A. Severe attacks with no response to therapy

B. Late stage

C. Early stage

D. Drugs induced asthma

Ans: A

Edited female slide 29

3- What do you advise a patient that has had bronchial asthma since childhood and positive family history?

A. Avoid exercise

B. Stop smoking

C. Avoid pollen grain

D. Avoid stress and anxiety

Ans: C

Male slide 5

4- Choose the best correlation?

A. Status asthmaticus / severe respiratory attack for days

B. Status asthmaticus / caused by Aspirin

C. Drug-induced / caused by workplace triggers

D. Intrinsic asthma / type I IgE-mediated hypersensitivity

Ans: A

Male slide 6 & 25

5- Which is the best state that describes an asthmatic attack ?

A. Induce drug asthma

B. Early developed asthma

C. Rhinitis

D. Sever attack with no response to the drugs

Ans: D

Male slide 25

6- Which statement is true about status asthmaticus ?

A. Its early phase of asthma

B. Severe, unrelenting attack

C. Drug induced

D. Responds quickly to treatment

Ans: B

Male slide 25

7- 17 years old boy with a strong family history of asthma with increased eosinophil, what advice would you give him?

A. Stop smoking

B. Avoid pollen grains

C. Avoid exercise and swimming

D. Avoid stress and anxiety

Ans: B

Male slide 5

8-A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing. The attacks are particularly common in the spring. During an episode of acute respiratory difficulty, a physical examination shows that she is afebrile. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. A sputum sample examined microscopically also has increased numbers of eosinophils. Which of

the following histologic features is most likely to characterize the lung in her condition?

E. Remodeling of airways with smooth muscle hyperplasia

F. Patchy areas of consolidation with leukocytic exudate in alveoli

G. Dilation of respiratory bronchioles with loss of elastic fibers

H. Inflammatory destruction of bronchial walls

Ans: A

Male slide 17 & 22

9-After taking a biopsy from a patient that undergoes an attack that lasts for weeks without response to treatment followed by hypercapnia and death after a few days . the microscope show?

I. Asteroid bodies

J. Hypertrophy of smooth muscle

K. Fibrosis

L. Goblet cells atrophy

Ans: B

Male slide 17

10-11 years old boy complains of cough, wheezing and difficulty of breathing, hyper resonance (increased air in lung), laboratory finding showed high IgE and eosinophil levels, what is the histological appearance of this acute condition?

a. Remodeling of airways with smooth muscle hyperplasia

b. Patchy areas of consolidation with leukocytic exudate in alveoli

c. Dilation of respiratory bronchioles with loss of elastic fibers

d. Inflammatory destruction of bronchial walls

Ans: A

Male slide 22

11- A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing, typically resulting from dust inhalation. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. Which of the following histologic features is most likely to characterize the lung in her condition?

A. Charcot-Leyden crystals

B. Curschmann spirals

C. Hyaline membranes

D. Hemosiderin-laden macrophages

Ans: A

Female slide 21

12- A 12-year-old girl has a 7-year history of coughing and wheezing and repeated attacks of difficulty breathing. The attacks are particularly common in the spring. During an episode of acute respiratory difficulty, a physical examination shows that she is afebrile. Her lungs are hyperresonant on percussion, and a chest radiograph shows increased lucency of all lung fields. Laboratory tests show an elevated serum IgE level and peripheral blood eosinophilia. A sputum sample examined microscopically also has increased numbers of eosinophils. Which of the following histologic features is most likely to characterize the lung in her condition?

A. Remodeling of airways with smooth muscle hyperplasia

B. Patchy areas of consolidation with leukocytic exudate in alveoli

C. Dilation of respiratory bronchioles with loss of elastic fibers

D. Inflammatory destruction of bronchial walls

Ans: A

Female slide 22

L4 - Interstitial Lung Disease

(6 Q's)

1- Regarding pneumoconiosis, which of the following are correct?

A. It is a restrictive lung disease

B. It is a disease of smokers

C. It is forced vital capacity is normal

D. It is forced expiratory volume markedly reduced

Ans: A

Female edit slide 4

2- What is seen in Late stage idiopathic pulmonary fibrosis?

A. Capillaries are filled with neutrophils

B. Alveoli are lined by type II pneumocytes and separated by collagen

C. Fibrinous exudate in pleura

D. Alveolar spaces are filled with foamy macrophages

Ans: B

Robbins Book, 11th edition page 411

3- A 50-year old female patient came in with Dry cough , Bilateral hilar lymphadenopathy with Non-caseating granuloma. Which of the following is the most likely diagnosis?

A. Sarcoidosis

B. Pneumoconiosis

C. Emphysema

D. Asthma

Ans: A

Female edit slide 26

4- A 60 year old man has dyspnea, nonproductive cough, he is a nonsmoker, he works as a shipbuilder and was exposed to asbestos 20 years ago. Which of the following is the most likely diagnosis?

A. Acute respiratory disaster syndrome

B. Good pasture syndrome

C. Malignant mesothelioma

D. Idiopathic

Ans: C

Female edit slide 12

5- Female african american has a non-caseating granuloma with skin lesion, what type of her erythema?

A. Erythema migrans

B. Erythema multiform

C. Erythema nodosum

D. Erythema toxicum

Ans: C

Robbins Book, 11th edition page 416

6- African females have high levels of calcium and ACE + bilateral hilar lymphadenopathy. What is the diagnosis?

A. Sarcoidosis

B. Silicosis

C. Berylliosis

D. Asbestosis

Ans: A

Female edit slide 26

L5 - Pathogenesis of pneumonia

(11 Q's)

1- A patient's lung autopsy showed liver-like lung and histologically the alveoli was filled with fibrin, red cells and neutrophils.

What stage of pneumonia is that?

A. Congestion

B. Red hepatization

C. Grey hepatization

D. Resolution

Ans: B

Female edite slide 13

2- An alcoholic patient, vomited multiple times a day.

Presented with cough with foul sputum. The cause of this could be?

A. Viral pneumonia

B. Pulmonary infarct

C. Aspiration pneumonia

D. Nosocomial pneumonia

Ans: C

Female edit slide 30

3- Lysis of RBC's and fibrin exudate in alveoli are seen in which stage of acute bacterial pneumonia ?

A. Red hepatization

B. Grey hepatization

C. Resolution

D. Congestion

Ans: B

Female edit slide 15

4- A patient came with Flu-like symptoms and cough and little amount of sputum , no consolidation . The X -ray showed Ground glass opacities under microscope the alveolar sample showed hyaline membranes. What is the cause?

A. Atypical viral pneumonia

B. Lung abscess

C. Acute bacterial pneumonia

D. Nosocomial pneumonia.

Ans: A

Female edit slide 32 & 33

5- 56 year old alcoholic man present to the clinic with foul smelling sputum later on an x ray was ordered which revealed a lung abscess what could be the cause of this abscess?

A. Aspiration of gastric content

B. Pulmonary embolism

C. Viral pneumonia

D. Tuberculosis

Ans: A

Female edit slide 27

6- Why are smokers at higher risk of infection than non-smoker ?

A. Impaired epiglottic reflex

B. Impaired mucociliary clearance

C. Impaired cough reflex

D. Cause aspiration

Ans: B

Female edit slide 3

7- A 72-year-old man presents with severe shortness of breath and has been diagnosed with acute bacterial pneumonia. Which of the following underlying conditions most likely increased his risk for developing this acute infection?

A. Pulmonary Fibrosis

B. Congestive Heart failure

C. Lung cancer

D. Pulmonary hypertension

Ans: B

Female edit slide 9

8- Which of the following is characteristic histologic feature in atypical pneumonia?

A. Alveolar spaces are filled with fibrin and blood

B. Alveolar spaces are clear from inflammation

C. Alveolar septa are clear from inflammation

D. Alveolar spaces are filled with neutrophils

Ans: B

Female edit slide 33

9- A 74 years old male presented to the emergency with lethargy , fever , productive cough , worsening dyspnea for a week duration . Diagnosis was demonstrate severe pneumonia. Which of the following is complication of bacterial pneumonia?

A. Lung cancer

B. Heart failure

C. Lung fibrosis

D. Empyaema

Ans: D

Female slide 3

10- Case about an HIV patient; on microscope they found cup-shaped cysts in silver stain, most common likely organism?

A. Histoplasma

B. Coccidioides

C. Pneumocystis jiroveci

D. Candidiasis

Ans: C

Female slide 41

11- (Case) A patient presents with symptoms of dyspnea, cough, and malaise. She has not traveled recently. Laboratory tests indicate leukocytosis with an abundance of neutrophils and hemoptysis. A chest X-ray reveals consolidation in the right middle lobe.

A. Primary tuberculosis

B. Pneumonia

C. Bronchitis

D. Asthma

Ans: B

Female slide 9 & 18

L6 - Diseases of pulmonary vasculature

(10 Q's)

1- Lines of Zahn are seen in:

A. Pulmonary congestion

B. Arterial thrombi

C. Postmortem clot

D. Venous thrombi

Ans: B

Female slide 40

2- A 25-year-old male presented to the ER with a 2-day history of cough, fever, shortness of breath, and weight loss. He had visited a patient with similar symptoms. What's the most likely diagnosis?

A. Tuberculosis

B. Pneumonia

C. COPD

D. Bronchial asthma

Ans: A

Female slide 8

3- Pulmonary embolism will cause hypoxemia by which of the following ways?

A. Increase V/Q ratio around embolus

B. Decrease V/Q ratio distal from embolus

C. Decreased shunting

D. Edema and defect diffusion

Ans: A

Female slide 32

4- A 30-year-old female with a history of renal transplant and chronic coughs presented with dyspnea. Upon examination, the lung had nodules. On biopsy, intact spherules within multinucleated giant cells were found. What is the causative microorganism?

A. Tuberculosis

B. Histoplasma

C. Coccidioides

D. Blastomyces dermatitidis

Ans: C

Female slide 17

5- A 36-year-old woman came with shortness of breath. She had a history of a clot and was not taking any preventative measures. If she develops pulmonary embolism, what physiological change will happen?

A. Decrease respiratory rate

B. Decrease heart rate

C. Hypoventilation

D. Increase alveolar dead space

Ans: D

Female slide 36

Dead space—>no gas exchange

And due to the embolism, there will not be any blood going to the alveoli so there will not be any gas exchange that's why there will be increasing in dead space.

6- What microorganism causes pulmonary mycosis characterized by broad budding on a silver stain?

A. Histoplasma

B. Coccidioides

C. Blastomyces

D. D.Actinomycosis

Ans: C

Female slide 17

7- A 45-year-old obese male came to the ER with chest pain and shortness of breath. After measuring the vital signs, his blood pressure was high. What is a significant contributor to the development of cor pulmonale?

A. Recurrent thromboemboli

B. Congenital or acquired heart disease

C. Interstitial lung diseases

D. Obstructive sleep apnea

Ans: D

Female slide 26

We have to choose something that cause pulmonary hypertension

8- A 57-year-old patient came to the ER with pulmonary hypertension. What is the most common change that occurs in his arteries?

A. Elastic degeneration

B. Medial hypertrophy

C. Thinning of the vessel wall

D. Arterial dilation

Ans: B

Female slide 32

9- A 25-year-old Indian male presented to the hospital with a 2-day history of fever, cough, and hemoptysis. He stated that he was previously healthy. A CXR shows a cavity lesion in his lung. What is the cause of his condition?

A. Pulmonary embolism

B. Pulmonary tuberculosis

C. Lung abscess

D. D.Blastomyces dermatitidis

Ans: B

Female slide 12

10- What microorganism causes pulmonary mycosis characterized by broad budding on a silver stain?

A. Histoplasma

B. Coccidioides

C. Blastomyces

D. D.Actinomycosis

Ans: C

Female slide 17

L7 - Tumors of the lung

(6 Q's)

1- A group of large cancer cells with bizarre anaplastic cells:

A. Large cell carcinoma

B. Small cell carcinoma

C. Squamous cell carcinoma

D. Adenocarcinoma

Ans: A

Female slide 20

2- A 45 year old Female patient presented with a peripheral tumor Microscopy showed malignant cells forming glands

A. Active mutation in EGFR gene

B. Active mutation in BAP1 gene

C. Active mutation in Tp53 gene

D. Deletion in 3p

Ans: A

Female slide 15

3- A smoker came to clinic with tumor in central lung tissue , under microscope small round,cells with scanty cytoplasm ,finely granular chromatin (salt & pepper). what is the correct about this tumor?

A. Neuroendocrine origin

B. Late local invasion.

C. Easy to prognosis

D. Takes time to metastasis lymph node at time of diagnosis.

Ans: A

Female slide 23

4- 50 year old female , 4cm apical malignancy with enophthalmos, ptosis, meiosis,and anhidrosis she has?

A. Horner's syndrome

B. Hyperparathyroidism

C. Turner syndrome

D. Hyperthyroidism

Ans: A

Female slide 28

5- The percentage of lung cancer, because of smoking

A. 80-90%

B. 50-60%

C. 10%

D. 100%

Ans: A

Female slide 8

6- A 66-year old man, resident of Mumbai who used to work in Shipyard, was referred to the outpatient department complaining of predominantly dry cough and dyspnea on exertion. Chest computed tomography (CT) revealed heterogeneously enhancing nodular pleural thickening. Histopathology revealed malignant biphasic neoplasm showing both Epithelial and Sarcomatoid components. Which of the following is most likely involved in the pathogenesis of this tumor?

A. Arsenic

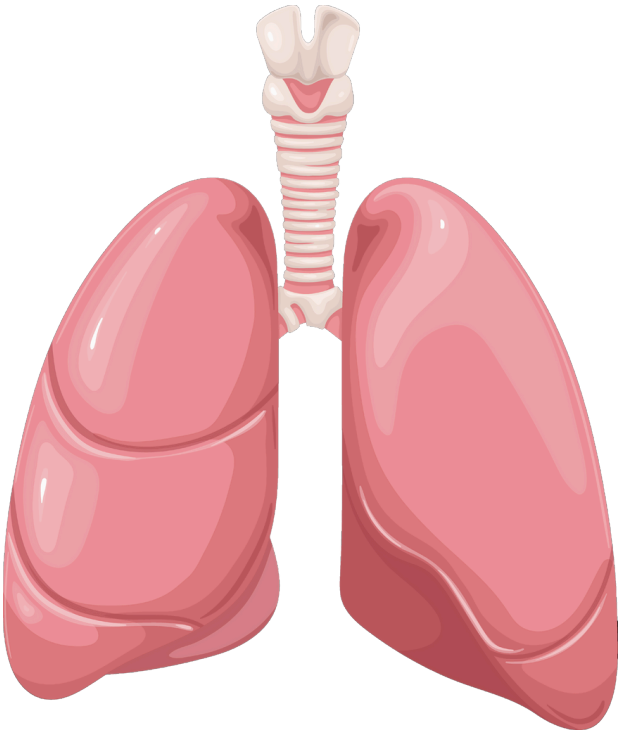
B. Carbon dust

C. Chromium

D. Asbestos

Ans: D Female edit slide 34

اللهم ذكرني ما نسيت و علمني ما جهلت



MEDICINE 

L1- Clinical approach to LRTI

(1 Q's)

1- If you suspect pneumonia in a patient, auscultation would reveal which of the following?

A. Wheezing

B. Bronchial breath sounds

C. Egophony

D. Friction rub

Ans: B

Male slide 9

L2- clinical approach of pleural diseases

(3 Q's)

1- Pneumothorax clinical signs are associated with all of the following except:

A. Hyper resonance to percussion

B. Decreased breath sounds

C. Decreased chest expansions

D. Crepitation's

Ans: D

Male slide 34

It is not mentioned clearly in the slide, but it is important to know the difference between clinical presentation of pneumothorax and pneumonia in the first lec.

The Crepitation and Crackels sounds are associated with pneumonia

2- A 30 y/o woman represents with signs and symptoms of plural effusion. You decided to do diagnostic thoracocentesis. For what —- if the effusion is transudative or exudative?

A. RBCs content
B. Protien and LDH level
C. Cell count
D. WBCs content

Ans: B

Male slide 20

3- A 45 years business woman came to ER with cough hemoptysis (tackycardia) tackypenea what is the diagnosis you will rule out ?

A. Plueral effusion
B. Acute coronary syndrome
C. Pneumonia
D. Pulmonary embolism

Ans: A

Male slide 11

4-Which of the following will yield a transudate fluid?

A) Malignancy

B) Heart failure

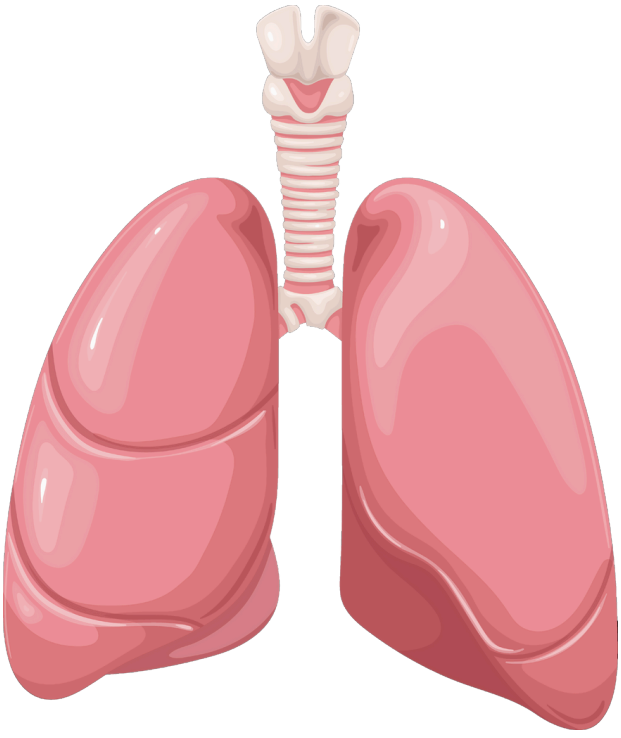
C) Tuberculosis

D) Pneumonia

Ans : B

Female slide 24

اللهم يا جامع النّاس في يوم لا ريب فيه
اجمع عليّ ضالتي



RADIOLOGY

L1- Radiology of the Lungs I

(5 Q's)

1- Posterioanterior in thorax radiogram means?

A. The patient laying on his back

B. The patient is in sitting position

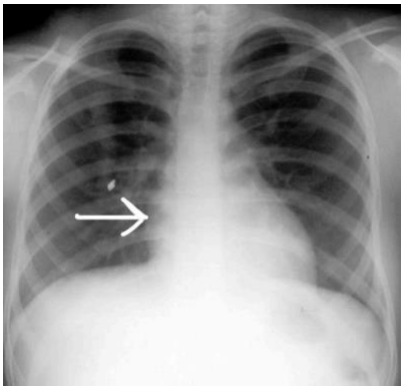
C. The patient is slightly rotated

D. The patient's back is on the x Ray tube

Ans: D

Female edit slide: 3

2- Identify the marked structure?



A. Right atrium

B. Pulmonary artery

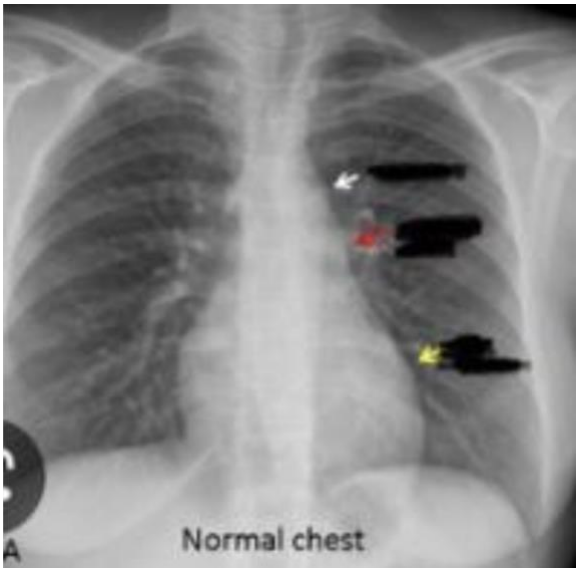
C. Pulmonary vein

D. Left atrium

Ans: A

Female edit slide: 27

3- Identify the structure marked in the red arrow?



A. Pulmonary artery

B. Pulmonary vein

C. Left atrium

D. Right atrium

Ans:A

Female edit slide: 27

4- What is the structure?



A. Aortic arch

B. Right pulmonary artery

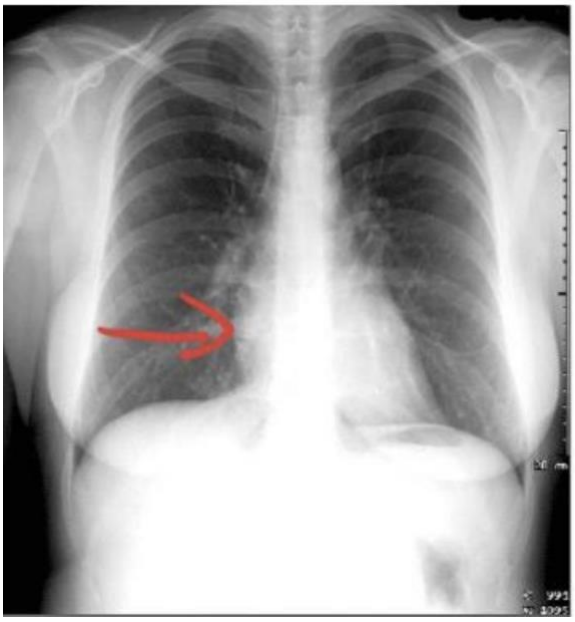
C. Right hilum

D. Right atrium

Ans:A

Female edit slide: 27

5- Identify the marked structure?



A. Right hilum

B. Right ventricle

C. Right pulmonary artery

D. Right atrium

Ans:D

Female slide: 27

L2- Radiology of the Lungs II

(8 Q's)

1- The hallmark of radiological appearance of lung collapse on chest radiograph:

A. Movement of normal anatomical structure

B. Air Bronchogram sign

C. Heterogeneous opacity with no significant mass effect

D. Contralateral deviation of the mediastinum

Ans: A

Male Slide 7

2- In Lung collapse:

A. Volume increases
B. Movement of anatomical structures
C. Air Broncho gram sign
D. Contralateral deviation of the mediastinum

Ans: B

Male Slide 7

3- In lung collapse, the radiological sign that can be seen is?

A. A.V shape costophrenic angle
B. B.Movement of normal anatomic
C. U shape costophrenic angle

Ans: B

Male Slide 7

4- The radiological sign that is seen with lobar pneumonia is:

A. Air-Broncho gram
B. Shifting of the mediastinal structures

C. U-shaped cost phrenic angle

D. V-shaped costophrenic angle

Ans: A

Male Slide 5

5- A patient with shortness of breath and, dyspnea visits the doctor. Upon Xray we find the trachea is deviated to the left and consolidation in the right lung . What does this indicate

A. Collapsed Left Lung

B. Collapsed Right Lung

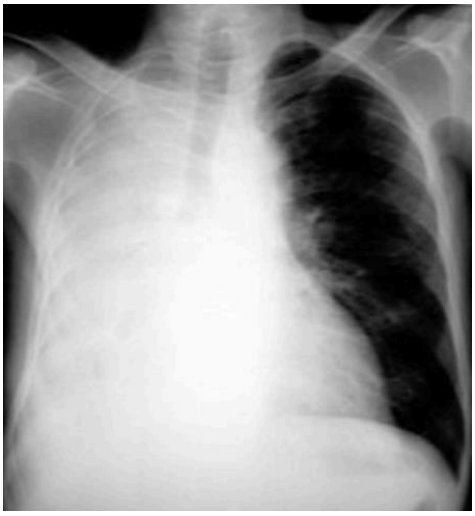
C. Massive Pneumothorax of the right lung

D. Massive Right lung pleural Effusion

Ans: D

MED22 male edit slide: 50

6- What is the abnormality in this picture?



A. Right side pleural effusion

B. Right side pneumothorax

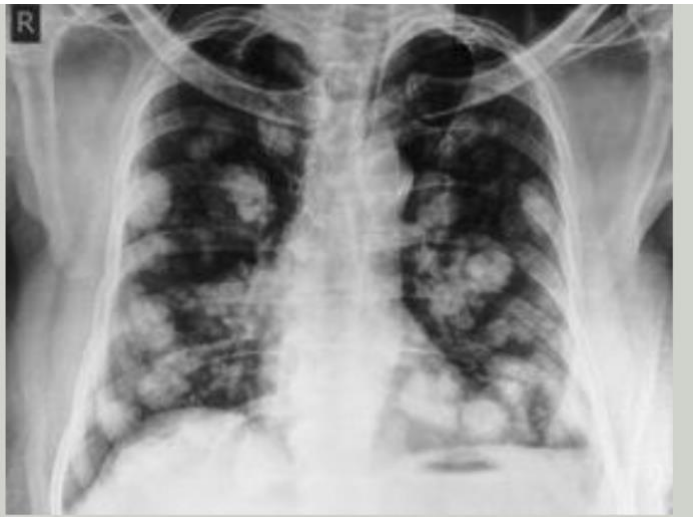
C. Right side collapse

D. Left side pneumonectomy

Ans: C

MED22 male edit slide 51

7- This x-ray show?



A. Miliary TB

B. Cannon Ball Metastasis

C. Cavities

D. Lung consolidation

Ans: B

MED22 male edit slide: 61

8- What is the abnormality in this picture



A. Left pleural effusion
B. Left pneumothorax
C. Left collapse
D. Left pneumonectomy

Ans:A

MED 22 male edit

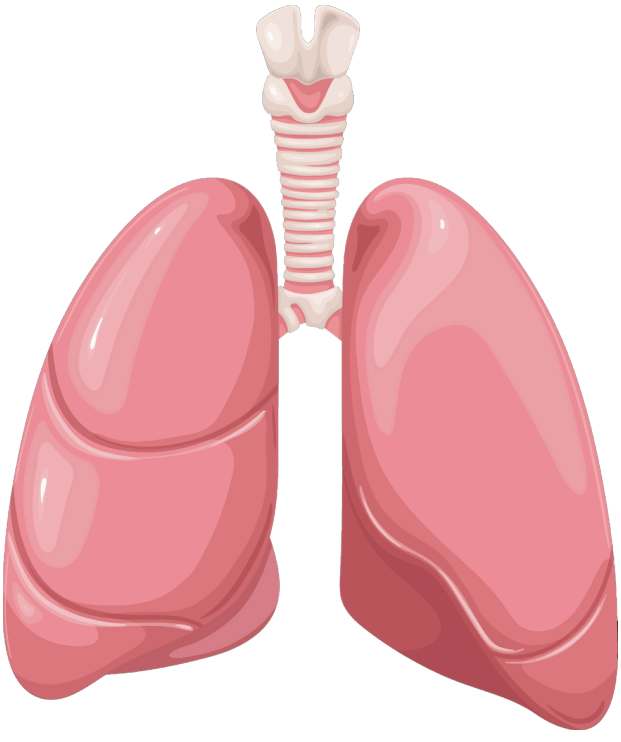
اللهم اجعل ألسنتنا عامرةً بذكرك،

وقلوبنا بخشيتك،

وأسرارنا بطاعتك،

إنّك على كلّ شيء قدير،

وحسبنا اللَّه ونعم الوكيل.



PHARMACOLOGY



L1- Drugs used in lung infection

(17 Q's)

1- A 32-year-old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?

A. Levofloxacin

B. Ampicillin with Sulbactam

C. Cefuxime

D. Penicillin

Ans: A

Female slide 23

2- A child was infected and upon blood culture the Gram + causative agent showed beta hemolytic colonies. The most suitable treatment is?

A. Penicillin

B. Ampicillin

C. Erythromycin

D. Clindamycin

Ans: A

Female slide 9

3- The mechanism of action of Doxycycline?

A. Inhibit bacterial protein synthesis

B. Inhibit bacterial topoisomerase

C. Inhibit bacterial

D. Inhibit bacterial wall synthesis

Ans: A

Female slide 30

4- In case of Mycoplasma pneumoniae infection, the suggested treatment will be?

A. Vancomycin

B. Cephalosporins

C. Penicillin

D. Erythromycin

Ans: D

Female slide 27

5- A 67 years old man has fever and cough, 3 days ago, he suffered from nasal congestion and sore throat, productive cough, the X-ray showed pulmonary infiltrate left lower lobe, what is the antibiotic choice for treatment?

A. Amoxicillin, clavulanic acid

B. Levofloxacin, gentamicin

C. Doxycycline

D. Ceftriaxone

Ans: A

Female slide 9&10

6- A 35 years old pregnant lady visited the clinic with a mild fever and scanty sputum, CXR showed diffuse bilateral infiltration. What is the suggested drug for this patient?

A. Azithromycin

B. Doxycycline

C. Levofloxacin

D. Cephalexin

Ans: A

Scanty sputum indicates atypical pneumonia, so either A or B, but B is teratogenic and is not used in pregnancy

7- A 32-year-old male has community acquired pneumonia, it was noted that he is allergic to ampicillin, which of the following is the treatment of choice?

A. Levofloxacin

B. Ampicillin with Sulbactam

C. Cefuxime

D. Penicillin

Ans: A

Female slide 23

8- A 4-year-old child presented with fever, runny nose, and inspiratory whoop following bouts of coughing. What is the drug of choice?

A. Erythromycin

B. Ciprofloxacin

C. Cephalosporin

D. Tetracycline

Ans: A

Female slide 28

9- Which of the following antibiotics is impaired with calcium?

A. Tetracycline

B. Macrolides

C. Amoxicillin

D. Cephalosporin

Ans: A

Female slide 34

10- A 75-year-old patient with a history of diabetes and chronic heart disease suffered from severe pneumonia. What is the first-line drug used for empirical treatment?

A. Ceftriaxone

B. Ciprofloxacin

C. Amoxicillin

D. Azithromycin

Ans: A

Female slide 10

11- A young adult came in with community acquired pneumonia, they have no history of illness, they described a history of resistance to penicillin, what is the best drug to give them?

A. Levofloxacin

B. Cefuramine

C. Ceftriaxone

D. Ampicillin

Ans: A

Female slide 23

12- Which drug binds to penicillin-binding protein?

A. Macrolides

B. Tetracycline

C. Beta lactam

D. Fluoroquinolones

Ans: C

Female slide 13

13- Which drug is given to overcome the resistance of beta-lactamase-producing bacteria?

A. Ciprofloxacin

B. Clavulanic acid

C. Doxycycline

D. Piperacillin

Ans: B

Female slide 12

14- A 28-year-old pregnant woman, currently in her second trimester, presents to the emergency department with fever and a respiratory rate of 27 breaths per minute. Upon examination, diffuse bilateral infiltrates are noted on her chest X-ray. Given her condition, what is the most appropriate antibiotic treatment?

A. Azithromycin

B. Dofloxacin

C. Doxycycline

D. Levofloxacin

Ans: A

Female slide 29

15- A patient with severe urinary tract infection is prescribed amoxicillin. Given the pharmacokinetics of this drug, which of the following should the doctor be aware of regarding the patient's state?

A. History with penicillin allergy
B. Renal impairment
C. Hepatic enzyme induction
D. Gastric pH alterations
E. CYP450 interactions

Ans: B

Female slide 17

16- Adverse effects of Doxycycline:

A. Photosensitivity
B. Respiratory Depression
C. Arrhythmias
D. Superinfection

Ans: A

Female slide 36

17- Azithromycin adverse side effect:

A. Prolonged QT interval
B. Gastrointestinal problems
C. Ototoxicity
D. Photosensitivity

Ans: A

Female slide 30

L2- Drugs used in bronchial asthma

(24 Q's)

1- Which of the following drug affects growth because it's have been taken a lot during childhood?

A. Corticosteroids

B. Ampicillin

C. Theophylline

D. Macrolides

Ans: A

Female slide 32

2- A 26 year old With history of bronchial asthma maintained by Salbutamol had an asthma attack without response to Salbutamol called the emergency in this situation. What should be done?

A. Start on Theophylline and to go to the emergency room

B. Discontinue Salbutamol and take Ipratropium

C. Take corticosteroids and go to the emergency room

D. Double the Salbutamol and go to the emergency room

Ans: C

Female slide 27

3- A female came to the ER with insomnia and seizures after taking an overdose of a direct bronchodilation drug which is used orally usually for asthma attacks ?

A. Ipratropium
B. Prednisone
C. Theophylline
D. Cromolyn

Ans: C

Female slide 25

4- A patient who has a cough and wheezing. He take inhaling corticosteroid what is the instruction that he should follow ?

A. Rinsing the mouth after inhalation with water
B. Holding the breath for 2 minutes after inhaling
C. Inhaling rapidly and forcefully for maximum effect
D. Using the inhaler immediately before meals

Ans: A

Female slide 31

5- Which of the following drugs have tolerance to its own therapeutic effects?

A. Short acting beta2 agonists

B. Montelukast

C. Theophylline

D. Cromolyn

Ans: A

Female slide 19

6- Which of the following reduces the effectiveness of salbutamol?

A. Propranolol

B. Theophylline

C. Corticosteroid

D. Tiotropium

Ans: A

Female slide 42

7- A 70 kg woman came to the ER with acute exacerbated asthma. After setting an oxygen cannula, what is the first appropriate drug to be used for treatment?

A. Salbutamol nebulization

B. Slow IV Cortisone

C. Aminophylline slow IV infusion

D. Ipratropium nebulization

Ans: A

Female slide 18

8- During pregnancy which of the following drug preferred in long term asthma control ?

A. Albetrol

B. Theophylline

C. Theobromine

D. Inhaled corticosteroids

Ans: D

Female slide 32

9- Which of the following drugs worsens the Asthma control ?

A. Avorostatin

B. Propranolol

C. Cephalosporin

D. Losartan

Ans: B

Female slide 42

10- What happens when we use beta 2 agonist with regular use ?

A. Tolerance
B. Increased risk of asthma attacks
C. Improved lung function permanently
D. Decreased heart rate

Ans: A

Female slide 19

11- Which of the following drugs are used in asthma and cause hoarseness of voice and candidiasis ?

A. Theophylline
B. Ipratropium bromide
C. Zileuton
D. Beclomethasone

Ans: D

Female slide 31

12- Work of breathing is decreased during ?

A. B2 agonist
B. Anti inflammatory drugs

C. Pregnancy

D. Bronchial asthma

Ans: A

Female slide 17

13- What is the pharma action of cromolyn ?

A. Beta agonist

B. Bronchodilator

C. Inhibits Mast cell degranulation

D. Bronchoconstriction

Ans: C

Female slide 33

14- What is the drug that is used mainly in acute asthmatic attack ?

A. Salbutamol

B. Cromolyn

C. Omalizumab

D. Corticosteroids

Ans: A

Female slide 18

15- Rinsing the mouth after inhalation of corticosteroids help to :

A. Increase the absorption of the medication

B. Enhance the bronchodilator effect

C. decrease the possibility of oropharyngeal candidiasis

D. Prevent the development of tolerance to the medication

Ans: C

Female slide 31

16- A 15 years old boy presented to ER with persistent, moderate asthma he uses Fluticasone 220 mcg (orally), 1 puff twice a day, Montelukast 10 mg daily, and Salbutamol (albuterol) when needed. After 3 months of well-controlled, the doctor will recommend:

A. Lower Montelukast dose

B. Lower Fluticasone dose

C. Increase Salbutamol frequency

D. Stop all medications

Ans: B

Inhaled corticosteroids like fluticasone are effective for controlling inflammation in asthma, but long-term use, especially at higher doses, can lead to side effects such as adrenal suppression, weight gain, and increased risk of infections. Since

the patient's asthma is well controlled, reducing the corticosteroid dose can help maintain control while minimizing these risks.

17- All the following cause asthma EXCEPT:

A. Beta blockers
B. Ibuprofen
C. Atropine
D. Exercise

Ans: C

Female slide 20

18- Omalizumab is best prescribed for:

A. Chronic asthma in patients >12 years as an add-on therapy with inhaled corticosteroids
B. Chronic asthma in patients >12 years as add-on therapy with LABA
C. Chronic asthma for patient >12 years and are uncontrolled with ICS (inhaled corticosteroid)
D. Acute exacerbation of chronic asthma

Ans: C

Female slide 38

19- All of the following increase the work of breathing except:

A. Beta antagonist

B. Anti-inflammatory drug

C. Bronchial asthma

D. Lying down

Ans: B

Female slide 29

20- Which of the following drugs can be used as a rescue drug for asthmatic attacks?

A. Ipratropium

B. Salbutamol

C. Cromolyn

D. Prednisolone

Ans: B

Female slide 18

21- Which of the following is used for treating a hospitalized patient with severe acute asthma?

A. Aminophylline slow IV infusion

B. Albuterol

C. Cromolyn

D. Montelukast

Ans: B

Female slide 18

22- Why is Theophylline considered a 3rd line of treatment in acute asthma ?

A. low bioavailability after oral
B. Narrow therapeutic index
C. A+B
D. None

Ans: B

Female slide 25

23- Which antibiotic is used before exercise and does not cause bronchodilation?

A. Inhaled cromolyn
B. Inhaled beta agonist
C. Montelukast
D. Oral corticosteroid

Ans: A

Female slide 33

24- Dosing for omalizumab is based on which of the following criteria :

A. Baseline severity of asthma

B. Renal function

C. Baseline of IgE level

D. Liver function

Ans: C

Female slide 38

L3- Drugs used in COPD

(6 Q's)

1- Ali is a 65 years-old man with COPD. He is currently using albuterol inhaler PRN (when required), Salmeterol inhaler twice a day, and tiotropium once a day. His COPD is still uncontrolled with frequent symptoms and recurrent exacerbation. What recommendation would you make to his regimen?

A. Add on oral corticosteroids once daily
B. Add on theophylline once daily
C. Change the ipratropium inhaler to ipratropium in scheduled four times a day
D. Change salmeterol to fluticasone/salmeterol combination in scheduled two times a day

Ans: D

Female slide 55

2- A 69 years-old man has COPD with moderate airway obstruction. Despite using salmeterol two times daily, he reports continued symptoms of shortness of breath with mild exertion. Which agent is an appropriate addition to his current therapy?

A. Salbutamol
B. Tiotropium
C. Systemic corticosteroids

D. Theophylline

Ans: B

Female slide 55 ,20, 21

3- A 68 year-old-male has COPD with moderate airway obstruction. Despite using salmeterol twice daily as prescribed, he reports continued symptoms of shortness of breath. Which one of the following agents would be an appropriate addition to his current therapy?

A. Systemic Corticosteroids

B. Albuterol

C. Tiotropium

D. Theophylline

Ans: C

Female slide 55 ,20, 21

4- Which of the following choices of drugs are preferred in COPD?

A. Theophylline and antihistamine

B. Beta agonist and Muscarinic antagonist

C. Beta antagonist and inhaled corticosteroids

D. Cholinergic agonist and inhaled corticosteroids

Ans: B

Female slide 49, 55,21

5- Which of the following is considered a mucolytic drug?

A. Stimulates the mucociliary activity of the respiratory epithelium
B. Reduces the mucous secretions
C. Increases the mucous secretions
D. Makes the respiratory secretions more watery

Ans: D

Female slide 52

6- Exacerbate chronic bronchitis and take oxygen but he needs drugs. What is initial therapy?

A. Nebulization salbutamol 2.5mg every 20 second
B. Nebulization Ipratropium 0.5mg every 20 second
C. Inhaled corticosteroids
D. Antibiotics

Ans: B

Female slide 21,48

L4- Drugs used in treatment of TB

(10 Q's)

1- The toxic effects of INH (central and peripheral neuropathy) are reversed by:

A. Folate

B. Pyridoxine

C. Cyanocobalamin

D. Thiamine

Ans: B

Female slide 19

2- A female is on an anti-tuberculosis drug and oral contraceptives. After a while, she felt sick and had a positive pregnancy result. Which drug caused her pregnancy?

A. Rifampicin

B. Isoniazid

C. Streptomycin

D. Ethambutol

Ans: A

Female slide 22

3- Which of the following anti-TB drugs causes visual disturbances, including slight color blindness and vision impairment?

A. Isoniazid
B. Ethambutol
C. Pyrazinamide
D. Rifampicin

Ans: B

Female slide19

4- Which of the following drugs can cause hepatotoxic effects in the treatment of TB?

A. para-Aminosalicylic Acid
B. Rifampicin
C. para-Aminosalicylic Acid + Rifampicin
D. Ethambutol

Ans: C

5- A patient is taking a drug to control his TB, which results in hyperuricemia. What drug is responsible for this side effect?

A. Isoniazid
B. Rifampin

C. Pyrazinamide

D. Streptomycin

Ans: C

Female slide19

6- An HIV patient recently diagnosed with TB started using HAART therapy (saquinavir). Which drug is least preferred?

A. Rifampicin

B. Isoniazid

C. Pyrazinamide

D. Ethambutol

Ans: A

Female slide22

Because it increase P450 which will increase the metabolism of these drugs

7- Choose the wrong statement:

A. Isoniazid causes decrease YP3A4 activity

B. Rifampicin causes increased CYP3A4 activity

C. Pyrazinamide is an inducer for CYP3A4

D. Warfarin effect is decreased by Rifampicin

Ans: C

Female slide22

8- Which drug's metabolism is affected by genetic differentiation in acetylation?

A. Isoniazid
B. Pyrazinamide
C. Fluoroquinolones
D. Ethambutol

Ans: A

Female slide20

9- A patient with DVT is currently on warfarin. He has recently been diagnosed with Tuberculosis, and the doctor wants to start first-line TB treatment. Which of the following is the appropriate action?

A. Review (increase) the warfarin dose
B. Stop taking Pyrazinamide
C. Increase the dose of Rifampicin
D. Decrease the dose of Isoniazid

Ans: A

Rifampicin induces liver enzymes → increases Warfarin metabolism → Warfarin becomes less effective → Need to increase/review Warfarin dose.

Female slide 14

10- A patient with DVT is currently on warfarin. He has recently been diagnosed with Tuberculosis, and the doctor wants to start first-line TB treatment. Which of the following is the appropriate action?

A. Review (increase) the warfarin dose
B. Stop taking Pyrazinamide
C. Increase the dose of Rifampicin
D. Decrease the dose of Isoniazid

Ans: A

(Rifampicin induces liver enzymes → increases Warfarin metabolism → Warfarin becomes less effective → Need to increase/review Warfarin dose.)

Female slide 14

L5- Pulmonary Embolism Drugs

(11 Q's)

1- Which of the following is incorrect about warfarin?

A. Its overdose can be reversed by vitamin k

B. It cannot cross placenta

C. Broad spectrum antibiotics can increase its action

D. Phenobarbitone can decrease its action

Ans: B

Female slide 14

2- Which of the following are direct thrombin inhibitors?

A. Argatroban

B. Dalteparin

C. Enoxaparin

D. Danaproid

Ans: A

Female slide 18

3- Which of the following is not an advantage of low molecular weight heparin?

A. It lowers the incidence of heparin induced thrombocytopenia
B. It enhances clot degradation
C. Lowers the frequent dosing
D. No need for routine monitoring of (aPTT)

Ans: B

Female slide 10 (equal efficacy)

4- LMWH is different from high molecular weight heparin in:

A. It inactivates factor Xa completely and only partially inactivate IIa (thrombin)
B. Completely inhibits factor IIa (Thrombin), IXa, and Xa
C. Inhibits the synthesis of vitamin K-dependent clotting factors
D. Independent from anti-thrombin III and directly binds to active site of factor IIa (Thrombin)

Ans: A

Female slide 10, 18

5- A 25-year-old male is brought to the ER due to excessive bleeding after an accident. It is found that he overdosed on warfarin. What is the antidote?

A. Vitamin K
B. Vitamin C

C. Vitamin B12

D. Vitamin D

Ans: A

Female slide 15

6- A 32-year old female has a valvular heart disease and takes warfarin, She decided to follow a new diet with lots of green vegetables. How would that affect her ?

A. Vitamin K is increased which makes warfarin less efficient

B. Vitamin C is decreased which makes warfarin more efficient

C. Vitamin K is decreased which makes warfarin more efficient

D. Vitamin C is increased which makes warfarin less efficient

Ans: A

Female slide 14 (not directly written but overall increased vit K in diet affects warfarin)

7- Which of the following is one of the advantages of LMWH ?

A. Easily monitor using INT

B. Cheap

C. Safe in pregnancy

D. Fully response to sulfate

Ans: C

Female slide 7, 10

8- What is the antidote for warfarin overdose?

A. Vitamin K
B. Vitamin B12
C. Vitamin D
D. D.Vitamin A

Ans: A

Female slide 15

9- A 32-year old female has a valvular heart disease and takes warfarin, She decided to follow a new diet with lots of green vegetables. How would that affect her ?

A. Vitamin K is increased which makes warfarin less efficient
B. Vitamin C is decreased which makes warfarin more efficient
C. Vitamin K is decreased which makes warfarin more efficient
D. Vitamin C is increased which makes warfarin less efficient

Ans: A

Female slide 14 (not directly written but overall increased vit K in diet affects warfarin)

10- When would we expect to see the maximum anticoagulant effect of warfarin?

A. One day
B. Three days
C. Six hours
D. One week

Ans: B

Female slide 14, 15

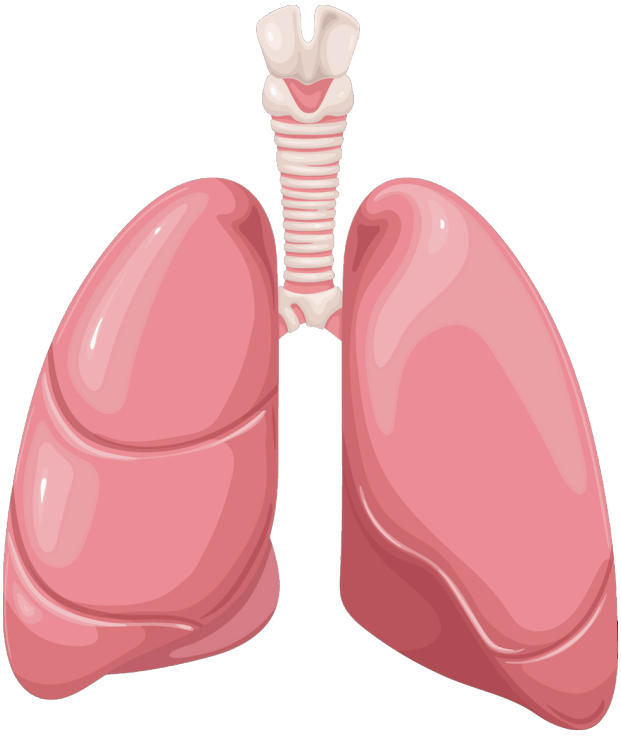
11- Which of the following will be used in case of heparin overdose?

A. Heparin sulfate
B. Protamine sulfate
C. Fresh frozen plasma
D. N-acetylcysteine (NAC)

Ans: B

Female slide 9

اللهم ذكرني ما نسيت و علمني ما جهلت



PARASITOLOGY

L1- Parasitic cysts of the lungs

(15 Q's)

1- The causes of anaphylactic shock?

A. Entamoeba histolytica

B. Hydatid cyst

C. Trematodes

D. Ascaris lumbricoides

Ans: B

Female slide 34

2- Infection with hydatid cyst occurs after?

A. Ingestion of parasitic cyst

B. Eating an organ with hydatid cyst

C. Ingestion of Echinococcosis granulosus egg

D. Inhalation of contaminated water

Ans: C

Female slide 27

3- A 37 year old Patient came with dyspnea,chest pain,and cough. ct scan and chest x-ray showed that there is pleural effusion and right lung abscess.what is the most causative agent?

A. Ascaris

B. Echinococcus garnulus

C. Entamoeba histolytica

D. Streptococcus pneumoniae

Ans: C

Female slide 15

4- A Patient go trip in alula and after two weeks he have upper right quadrant pain , jaundice, and abdominal mass with lung abscess. During surgery they find liver cyst. What the parasite that he infected with?

A. Echinococcus granulosus

B. Schistosomae

C. Entamoeba histolytica

D. Paragonimus westermani

Ans: A

Female slide 33

5- Larval form of Echinococcus granulosus is present in which host?

A. Dog

B. Wolf

C. Man

D. Sheep

Ans: C

Female slide 23

6- A patient presented with cough, dyspnea, chest pain. CXR/investigation revealed a small amount of pleural effusion and abscess in the lung. What is the most likely causative parasite?

A. Entamoeba histolytica

B. Echinococcus granulosus

C. Paragonimus westermani

D. Schistosomiasis

Ans: A

Female slide 15

7- A patient came to the ER with a cough and fever, X-rays showed elevated right diaphragm and lower right pleural effusion and abscess. What is the causative agent?

A. Entamoeba histolytica

B. Ascariasis

C. Hydatid cyst

D. Toxocariasis

Ans: A

Female slide 16

8- A patient came in with skin flushed, they found a ruptured hepatic cyst. What is the causative agent?

A. Echinococcus

B. Ascariasis

C. Toxocariasis

D. Entamoeba histolytic**a**

Ans: A

Female slide 33

9- What is the preventative measure for Echinococcus?

A. Removal of snails

B. Avoid contamination of hands, food, and water with dog feces

C. Wear preventive clothes (gloves - shoes)

D. Proper washing of raw vegetables

Ans: B

Female slide 38

10- A male shepherd from AlUla came complaining of hepatomegaly, upper right quadrant pain, jaundice, and a non-painful hepatic cyst was found. What is the likely causative agent?

A. Echinococcus

B. Entamoeba histolytica

C. Schistosomiasis

D. Ascaris lumbricoides

Ans: A

Female slide 33

11- Where are the larval forms of Echinococcus found?

A. Man

B. Dog

C. Cat

D. Wolf

Ans: A

Female slide 23

12- What is the common extrahepatic site of ambiosis?

A. Peritoneum

B. kidney

C. Spleen

D. Lung

Ans: D

Female 15

13- A patient comes to the clinic with a chronic cough, fever, and right-sided chest pain. Imaging shows an elevated right diaphragm, What is a likely parasitic cause?

A. Entamoeba histolytica

B. Schistosomiasis

C. Toxocariasis

D. Ascaris lumbricoides

Ans: A

Female slide 16

14- What's the infective stage of entamoeba histolytica?

A. Egg

B. Trophozoites

C. Cyst

D. Cercaria

Ans: C

Female slide 13

15- A 57 year old farmer presents with dyspnea, chest pain, and a chronic cough. CXR shows homogeneous opacities that almost fill up the right lung. Which of the following is the most likely diagnosis?

A. Amoebiasis

B. Ancylostomiasis

C. Toxocariasis

D. Echinococcosis

Ans: D

Female slide 34

L2- Parasites causing visceral larva migrates

(12 Q's)

1- Loffler's Syndrome in the lung is due to?

A. Sensitization to ascaris antigen

B. inflammation reaction against ascaris larva in the lung

C. viral infection in the lung

D. Bacterial infection in the lung

Ans: B

Female slide 13

2- Which of the following is a prevention of Ancylostoma?

A. Wear shoes

B. Wash vegetables

C. Wash hands

D. Proper sewage disposal

Ans: A

Female slide 25

3- Which one of the following infections lead to Cor pulmonale?

A. Toxocariasis

B. Hydatid

C. Amoebiasis

D. Schistosomiasis

Ans: D

Female slide 42

4- A 42 year old man returned two weeks ago from a five month trip to Ghana. During his stay he had frequent visits to Lake which included swimming in the lake. He was present with fever, cough wheeze myalgia headache and urticaria.stool analysis revealed 2-3 eggs of schistosoma mansoni. What is the stage?

A. chronic schistosomiasis

B. Cercarial penetration (invasion stage)

C. Cercaria (migration stage)

D. Acute Schistosomiasis (Katayama)

Ans: D

Female slide 39&40

5- Which of the following is the egg that causes Ascaris infection to humans by raw vegetables and contaminated water?

A. Fertilised egg

B. Unfertilized egg

C. Mature egg

D. Immature egg

Ans: C

Female slide 10

6- Patient presented with 3 weeks of cough, dyspnea, CXR revealed bilateral zone patch consolidation, eosinophilia and Ascaris larva in sputum, what is this patient have ?

A. Katameya fever

B. Liver abscess

C. Lung cyst

D. Loffler's syndrome

Ans: D

Female slide 14

7- Which of the following can lead to Cor pulmonale?

A. Schistosomiasis

B. Toxocariasis

C. Echinococcosis

D. Ascariasis

Ans: A

Female slide 42

8- A 6-year-old boy played with dogs on a school trip, later he came in with chest tightness,and bronchospasm. What is the possible organism?

A. Toxocara

B. Schistosoma

C. Ascaris

D. Ancylostoma

Ans: A

Female slide 26&30

9- Patient ate green salad and got a parasitic infection with increased eosinophilia?

A. Ascariasis

B. Schistosomes

C. Toxocariasis

D. Entamoeba histolytica

Ans: A

Female slide 10&15

10- A 20 years old girl ate green salad and weeks later developed cough, dyspnea, and asthma. What is the parasite?

A. Ascaris

B. Ancylostoma

C. Toxocara

D. Strongyloides

Ans: A

Female slide 8

11- A 42 years old farmer presents with dyspnea, chest pain, and chronic cough. Stool examination revealed no parasitic eggs. Additional tests revealed eosinophilia and leukocytosis. What is the most likely diagnosis?

A. Pulmonary hydatid cyst

B. Löffler syndrome

C. Schistosomiasis

D. Amoebiasis

Ans: B

Female slide 11

12- A 42 years old man returned two weeks ago from a five-month trip to Ghana, during which he frequently swam in a freshwater lake. He now presents with fever, cough, wheeze, myalgia, headache, and urticaria. Stool analysis revealed 2–3 eggs of Schistosoma mansoni. What is the stage of infection?

A. Chronic schistosomiasis

B. Cercarial penetration (invasion stage)

C. Cercarial migration stage

D. Acute schistosomiasis (Katayama fever)

Ans: D

Female slide 36

اللهم إني أستودعك ما قرأت وما حفظت،
وما تعلمت، فرده عند حاجتي إليه، إنك على كل شيء
قدير، حسبنا اللَّه ونعم الوكيل.

